SAMA

Safe AI in Medicaid Alliance

ABSTRACT

With an estimated 18.5 million people potentially subject to Medicaid community engagement requirements, and a condensed implementation period, states will need scalable approaches to reach Medicaid members and support them through verification processes. AI-enabled automation can play an important role in meeting this challenge.

INTELLIGENT MEMBER OUTREACH FOR HR1

An AI Guide

SAMA Intelligent Member Outreach Workgroup

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<u>Terminology Note</u>: For purposes of this guide, "AI-enabled communications" may include generative AI, conversational AI, machine-learning capabilities, and workflows that combine AI with conventional automation. Not every capability described in this guide requires AI.

1. Introduction

House Resolution 1 (HR1) presents a significant implementation challenge at a time when many state Medicaid agencies face constrained budgets, limited staff capacity, and competing modernization priorities. An accelerated implementation timeline, requirements that are new to Medicaid members, and a complex ruleset all contribute to its difficulty. Clearly communicating what the new requirements are, why they matter, and what members must do to comply will be essential. With an estimated 18.5 million potentially affected, states will need a comprehensive and well-coordinated communications strategy that combines automation with human support. Given the volume and complexity, leveraging AI in HR1 communications enables a scalable approach to reach millions of Medicaid members and the communities that support them.

2. The Case for Using AI: Member Pain Points

One of the primary pain points Medicaid members face is the complexity of eligibility requirements. This complexity heightens communication challenges. Information accessibility (reading levels, languages, modalities, low cognitive load) is difficult to achieve given the diversity of the population. Further, making sure members receive information timely compounds the issue as members can be transient, change contact information frequently, and lack access to communication devices.

For example, members frequently experience challenges related to retro-active coverage and application renewals, particularly when renewals must occur twice a year. Frequent renewals can create additional administrative burden for both members and staff, and system updates needed to support these requirements may not be implemented quickly.

Policy changes, such as the implementation of the new HR1 requirements, are a key pain point for members. When a policy change affects a member's benefit, the member needs to: a) be aware of the change, b) be educated on the change, c) act on the change (submit documentation, etc.), and d) verify that their actions were sufficient. Members also need timely access to the status of their verification so they can address outstanding requirements before deadlines are missed. Without reliable communication and verification channels, members may struggle to retain benefits even when they meet actual policy requirements.

Reaching members and stakeholders with accessible and timely communications helps alleviate the burden of eligibility and policy changes. AI support can improve the reach of these efforts.

3. AI Communication Use Cases for HR1

HR1 introduces broad new Medicaid requirements that create substantial administrative demand for states, members, providers, and other stakeholders. AI-powered communication can assist with these requirements across nine distinct use cases.

3.1 AI Use Cases

Use Case	Trigger Scenario	AI Communication Method
Education and Information	Members unaware of HR1 requirements	AI call agents and chatbots deliver HR1 information proactively
Process Completion Support	Members stalled in an in-process status	Chatbot or AI agent provides step-by-step guidance to complete required actions
Activating Non-Responsive Members	Members take no action on HR1 requirements	Outbound AI calls and texts via state-authorized channels with reminders and deadlines
Policy Translation and Simplification	Legislative language inaccessible to members, providers, and agencies	AI converts statutory text into plain-English FAQs and audience-specific summaries
Personalized Outreach	Members need targeted notice by program, geography, or eligibility status	Tailored SMS, email, portal messages, or letters tied to individual status and deadlines
Contact Center and Self-Service Support	High call volume strains contact center capacity	AI chatbots, virtual assistants, IVR prompts, and agent-assist tools provide 24/7 support
State Employee Knowledge Support	State workers need rapid access to HR1 policy	AI-linked knowledge base provides searchable access and real-time response suggestions
Member Journey Stress Point Relief	Friction at journey stages causes confusion and disenrollment	AI prompts mapped to key journey stages reduce friction and guide completion
Generate Physical Letters	Members without digital access require paper-based communication	AI drafts compliant physical letters per CMS statutory notice requirements (CMS, 2025)

Use Case 1: Education and Information

Current applications of AI for education provide tailored and highly available support, while reducing customer service volume. AI support can:

- Explain requirements, qualifying activities, and exemptions in plain language
- Direct members to required forms and helplines with multilingual and 24/7 support
- Inform members of deadlines at application and renewal

Use Case 2: Process Completion Support

AI-driven status triggers with action-prompting can automate high-volume compliance tasks for members who are stalled in their verification process. AI support can:

- Identify the member's status within the compliance workflow and deliver step-by-step instructions for uploading documentation or completing required forms
- Confirm successful submission and set expectations for next contact or escalate complex cases to human agents

Use Case 3: Activating Non-Responsive Members

A significant share at risk members do not respond to initial mail notices. AI support can:

- Deploy outbound AI calls and SMS to members who have not responded; extend outreach to authorized representatives where permitted under state policy
- Sequence reminders across voice, text, email, and document attempts for compliance

Use Case 4: Policy Translation and Simplification

HR1 contains dense statutory language inaccessible to most members. AI support can:

- Generate plain-English summaries of community engagement requirements, exemptions, and consequences of non-compliance

- Create FAQs tailored to members, providers, employers, and state agency staff

Use Case 5: Personalized Outreach

The 2023 FCC ruling provides additional flexibility for the use of automated texts and calls for enrollment-related communications. AI support can:

- Trigger tailored SMS and email based on individual deadlines, missing documents, or eligibility changes and customize content by program, geography, and language
- Send provider-specific bulletins on exemption processes and documentation requirements

Use Case 6: Contact Center and Self-Service Support

HR1 will generate a surge of inquiries to Medicaid contact centers. AI support can:

- Deploy AI chatbots for 24/7 self-service; use IVR to triage calls and provide real-time agent-assist suggestions during live member interactions
- Auto-generate post-call summaries and apply AI quality management to a significantly higher share of calls than manual review allows

Use Case 7: State Employee Knowledge Support

Front-line employees need fingertip access to verified HR1 policy. AI support can:

- Connect state-approved HR1 documents, CMS guidance, and FAQs in a single searchable interface for all eligibility workers
- Surface exemption criteria, documentation checklists, and appeal procedures on demand during member interactions

Use Case 8: Member Journey Stress Point Relief

The HR1 compliance path contains multiple friction points where members disengage. Portal login barriers and document submission issues are common triggers. AI support can:

- Map HR1 compliance journey stages, for example, “notice receipt, portal login, document upload, status check, confirmation” and identify friction points
- Deploy targeted AI prompts, chatbot support, and outbound reminders at each identified stage

Use Case 9: Generate Physical Letters

HR1 mandates initial outreach by regular mail with specific required content. Certain Medicaid populations rely on paper mail as their primary channel. AI support can:

- AI assists in drafting personalized compliance notices, deadline reminders, and document-request letters from member case data using state-approved templates and required content subject to appropriate legal, policy, accessibility, and quality review
- Generate accessible formats (large print, plain language, translations) and integrate with print-and-mail vendors for end-to-end automation

These nine AI communication use cases, deployed as a coordinated framework can help reduce unintended disenrollment, lower contact center burden, consistent and provide equitable access across language, geography, digital access, and literacy levels.

4. AI Communication Vehicles for HR1

Effective AI-powered communications under HR1 require multiple complementary channels that work together to support members throughout their interactions with Medicaid programs. This section proposes a layered framework in which self-service chatbots provide 24/7 informational support, AI agents assist with routine inbound and outbound communications, SMS and email campaigns deliver timely reminders, and contact center AI tools enhance human agent effectiveness.

4.1 AI Communication Vehicles

Vehicle	Primary Function	HR1 Application
Chatbots	Answer basic informational questions 24/7	Members learn about HR1 requirements, exemptions, and next steps
AI Agents	Outbound and inbound AI-powered calls	First-level handling of inbound questions; proactive outreach to non-responsive members
SMS / Email Campaigns	Deadline reminders, renewal notices, missing-document alerts	Drive timely action and improve response rates ahead of HR1 deadlines
Contact Center AI / Agent Assist	Agent assist, call summaries, knowledge search, call routing	Improve call quality and ensure consistent responses across agents handling HR1 inquiries

Vehicle 1: Chatbots

Chatbots are AI-powered conversational tools embedded on state Medicaid websites, member portals, and mobile platforms. Trained on state-approved resource materials, they answer basic questions from members at any time of day without requiring a live agent, reducing call center volume while ensuring consistent, accurate responses. *Chatbot HR1 Applications:*

- Answer questions about community engagement requirements, qualifying activities, and documentation
- Explain exemptions and exclusions in plain language and guide members to correct forms or portals
- Provide multilingual support and operate 24/7 to reach members outside standard business hours

State Examples

Louisiana's Medicaid chatbot, MARC, answers questions in English, Spanish, and Vietnamese around the clock, transferring members to a live helpline when issues require human support. Oklahoma's SoonerCare launched SoonerGuide to handle common Medicaid eligibility questions, reducing pressure on customer service staff. CMS launched an AI chatbot for internal staff use in December 2024, training thousands of employees and reporting positive early productivity outcomes.

Vehicle 2: AI Agents

AI agents are autonomous or semi-autonomous voice and messaging systems that handle both outbound and inbound communications. Unlike basic chatbots, they conduct multi-turn conversations, interpret member intent, execute tasks such as scheduling and status checks, and escalate to human agents when needed. *AI Agent HR1 Applications:*

- Outbound: Proactively contact members who have not responded to initial HR1 notices through state-authorized channels

- Inbound: Serve as first-level responder for questions about community engagement requirements, exemptions, or documentation
- Handle appointment scheduling, status inquiries, and document submission confirmations across voice, SMS, and chat

State Examples

Ohio Medicaid's Baby Bot demonstrates AI agent automation for high-volume tasks—automatically adding newborns to their mother's Medicaid case and saving weeks of county staff time. Minnesota's Driver and Vehicle Services deployed a virtual assistant handling citizen question in English, Spanish, Somali, and Hmong, freeing agents for complex cases.

Vehicle 3: SMS and Email Campaigns

Automated SMS and email campaigns deliver targeted, timely messages about HR1 requirements, deadlines, and required actions. Campaigns are personalized by program type, geography, eligibility category, and member status. *SMS and Email Campaign HR1 Applications:*

- Deadline reminders for community engagement reporting and redeterminations
- Missing-document alerts, provider bulletins, and policy updates
- Follow-up to returned mail when physical notices are undeliverable

State Examples

A Louisiana pilot sending SMS renewal reminders produced a 10-percentage-point increase in Medicaid renewals and a 19-percentage-point increase in SNAP renewals. Iowa HHS operates a Medical Assistance Text Messaging Program sending eligibility notifications and renewal reminders to members' phones via opt-in enrollment. Montana uses automated SMS to follow up when mail is returned undeliverable.

SMS Regulatory Considerations:

- The FCC's 2023 rules provide additional flexibility for state Medicaid agencies to use automated text messaging for enrollment-related communications.
- Messages should use plain language, avoid unencrypted health information, include an opt-out mechanism, and be sent at the direction of the state agency.

Vehicle 4: Contact Center AI and Agent Assist

Contact center AI and agent assist tools augment human agents in real time with knowledge search, recommended answers, call summaries, compliance prompts, and intelligent call routing. State contact centers face dramatic increases in call volume under HR1 while managing workforce constraints. These tools do not replace agents, they make them faster, more consistent, and more confident. *Contact Center AI and Agent Assist HR1 Applications:*

- Surface relevant HR1 policy information and member-specific data during live calls without requiring agents to navigate multiple systems
- Generate post-call summaries automatically, freeing agents from manual documentation
- Route members intelligently and review a substantially larger share of calls for compliance than manual processes allow

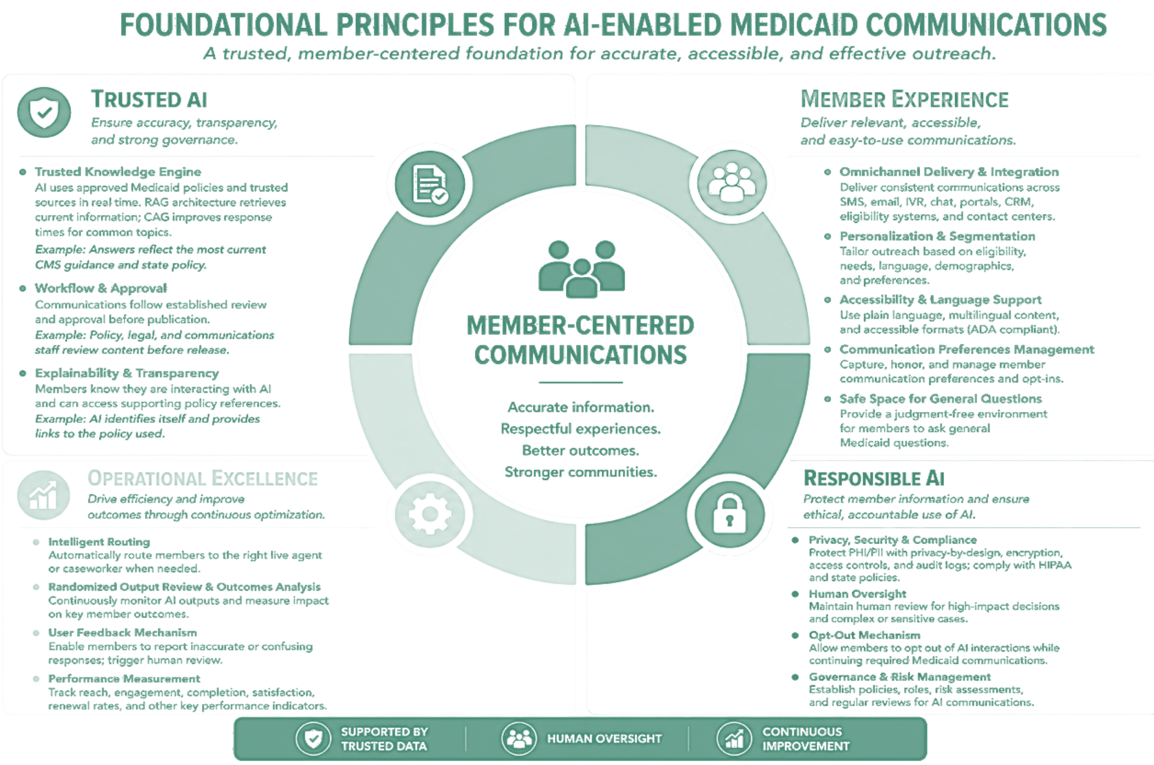
State Examples

Minnesota's Driver and Vehicle Services contact center demonstrated that routing routine inquiries to a virtual assistant allows human agents to focus on complex cases. AI-powered quality management enables agencies to evaluate a substantially larger share of member interactions than manual review permits, strengthening compliance monitoring and agent coaching.

5.0 Foundational Principles for AI Communications

Successful implementation of AI-enabled communications requires more than simply adopting AI technology. Agencies should establish a common set of foundational principles that promote accurate information, transparency, accessibility, operational efficiency, and responsible use of AI.

The principles outlined in the graphic below provide a practical framework for evaluating and implementing AI-enabled member communications while keeping the needs of Medicaid members at the center of every interaction.



This framework summarizes the foundational principles for designing trusted, member-centered AI communications. Together, these principles provide a practical model for evaluating AI communication solutions that improve member engagement while supporting accuracy, transparency, compliance, and operational efficiency.

5.1 Recommendations for Building an AI Communications Foundation

- Adopt a common framework for evaluating AI communication capabilities
- Maintain human oversight for high-impact or case-specific communications
- Establish governance processes that ensure communications remain accurate, current, and policy compliant
- Design communications that are accessible, multilingual, and centered on member needs
- Measure outcomes and continuously improve AI performance using analytics and member feedback

Artificial intelligence should augment—not replace—the expertise and judgment of Medicaid staff. When implemented using the foundational principles described in this section, AI can become a trusted tool that enhances member communications while preserving the human-centered approach that is essential to public health programs.

6. Guardrails For AI Use in Member Communications

AI-enabled outreach can expand capacity and improve access, but it can also distribute incorrect information at scale, expose sensitive data, or create barriers between members and state staff.

This section focuses on guardrails specific to AI-enabled Medicaid member outreach, including what AI should and should not do, when human review is required, and how member-facing interactions should be controlled and monitored. Broader enterprise AI considerations—including privacy, security, data protection, and compliance—are addressed elsewhere in this guide.

6.1 Keep Members and Member Rights at the Center

AI should support member understanding and actions, not make decisions affecting benefits. The key boundary is between communication support and actions that require legal authority, program judgment, or accountable human discretion.

What AI Should and Should Not Do

✓ AI supporting activities within approved parameters	✗ AI should not independently
-Explain approved information and simplify communications	-Determine eligibility or noncompliance
-Provide reminders and support process navigation	-Approve or deny exemptions
-Direct members to appropriate assistance	-Reject documentation or issue adverse actions
-Help staff retrieve, summarize, or route approved information	-Resolve appeals or grievances
	-Prevent or delay access to human assistance

AI may help a member understand what is required or locate the right resource, but it is not a replacement for Medicaid staff, who remain a key information resource. Additionally, qualified staff and established state processes must remain responsible for determinations, exceptions, adverse actions, and remedies that affect coverage or member rights.

6.2 Human Review and Mandatory Escalation

States should define which interactions require staff involvement and where AI may safely assist. Human escalation should occur whenever the interaction requires judgment, involves a significant risk to coverage or rights, or cannot be resolved reliably from approved sources. Escalation conditions should include:

- Potential loss of coverage or other adverse consequences
- Medical frailty, disability, accommodation needs, or good-cause questions
- Appeals, fair hearings, complaints, or grievances
- Complex household circumstances or conflicting case information
- Member distress, repeated confusion, or repeated failed attempts to resolve a concern

- Questions that cannot be answered reliably from approved sources

The handoff should preserve relevant context so that members are not required to restart the interaction or repeat information unnecessarily. AI should also recognize its limits. When it lacks an approved basis for answering, the appropriate response is to acknowledge that limitation and route the member to qualified assistance.

6.3 Transparency, Choice, and Human Access

Members should understand when they are interacting with AI and what alternatives are available. States should provide disclosure appropriate to the channel, explain the purpose of the AI interaction in simple terms, and offer a path to a human agent. Where appropriate, members should be able to decline AI-enabled interactions without being led to believe that they may opt out of legally required Medicaid notices or other required communications.

6.4 Language, Accessibility, and Equity

Communications should be understandable and usable by members with different languages, disabilities, literacy levels, and access limitations. States should support preferred languages, plain language, screen readers, large-print formats, relay services, and paper-based alternatives. AI-enabled outreach should supplement, not eliminate, communication methods needed by members who cannot reliably use digital channels.

6.5 Monitoring and Auditability

States should retain evidence to evaluate AI-enabled communications. Records should identify what communication was generated, approved source information, policy version, human interaction, escalation occurrences, member actions and complaints, and errors.

Effective guardrails help ensure that AI remains aligned with its intended purpose, supports members and staff, and does not exceed the role assigned to it.

7. Community Stakeholders and HR1 Communications

A well-thought-out HR1 communication strategy will include community stakeholders, such as providers, health plans, community organizations and social media groups. **Including these third parties in your plan offers advantages:**

- *They meet members where they are at health centers, community centers, and online*
- *They have language and cultural competency skills to communicate effectively*
- *They are trusted and have an established influence on member actions*

***Commun*ity stakeholders need to understand HR1 requirements** and be able to educate members and reinforce accurate state-approved information. States can educate community stakeholders on HR1 requirements through a) state-sponsored information sessions, b) HR1 liaisons, and c) stakeholder access to online tools and resources.

7.1 Use of AI in community stakeholder communications

AI can play an essential role in providing accessible support to community stakeholders. Where AI is deployed, care should be taken that the AI is trained with proper state and federal resource material. States should consider providing community stakeholders access to verified, state-approved AI tools, including chatbots that provide on-demand HR1information from authoritative sources.

7.2 Examples of HR1 community stakeholder engagement

Community Stakeholder	Why Engage Them	Methods
Community Based Organizations / Community Groups	They have access to many high-risk and hard to reach members, especially ones where English is not their primary language. Application of language and cultural considerations in communications enable awareness and trust in the process.	• HR1 community information sessions • Use of listservs and social media to spread awareness • Webinars and other digital information sessions • Awareness content posted in facilities and on website
Social Media Groups	They can have a high level of engagement with and influence over Medicaid members.	• HR1 awareness social media campaigns • Posting state-sponsored content and resources on their sites • Using AI to analyze member comments identify a lack of understanding or problems with the HR1 roll-out process
Providers / Health Systems	They are on the front lines of eligibility processes and are often a trusted source of information.	• Providing HR1 information during Intake and care coordination processes • Posting HR1 information in facilities and on website • Identifying Medical Frailty exclusions and informing members of exclusion process
Health Plans	They will coordinate with states on HR1 implementations and will be especially helpful with their members who actively engage with caseworkers.	• Proactively providing HR1 information during caseworker calls with members • Making HR1 information available at community events and local offices • Using their websites and social media to spread awareness and access to resources

Community stakeholders can be an essential part of HR1 implementations, making the process more efficient and reducing member confusion and reducing benefit loss. To leverage these groups successfully,

8. Data Strategy for AI-Enabled Member Communications

Effective AI-enabled member communications depend on trusted, accurate, and well-governed information. Whether members interact through a chatbot, contact center, text message, email, mobile application, or traditional paper correspondence, communications should be based on current policy and personalized using reliable operational data.

HR1 increases the importance of timely, accurate, and consistent member communications across channels. Successfully implementing these requirements requires more than new technology, states also need a governed data strategy.

8.1 Four Core Data Domains

A trusted data strategy is built on four core data domains supported by enterprise governance.

Domain 1: Trusted Policy Knowledge	Domain 2: Operational Member Data	Domain 3: External Verification Data	Domain 4: Analytics and Performance Measurement

Domain 1: Trusted Policy Knowledge

Trusted policy knowledge provides the authoritative content used in member communications. Federal and state regulations, policy manuals, operational guidance, approved FAQs, member notices, and call center scripts should be maintained as a single source of truth to ensure communications remain accurate and consistent as policies evolve.

Domain 2: Operational Member Data

Policy defines the content of a communication; operational data determines who should receive it, when it should be delivered, and through which communication channel.

Operational data includes eligibility information, work requirement status, MMIS information such as medical frailty and exemptions, communication preferences, communication history, and case management information. States should retrieve only the minimum information necessary to support a member interaction while protecting privacy and maintaining compliance.

Domain 3: External Verification Data

States should maximize the use of federal, state, and approved commercial verification sources before requesting documentation from members. Leveraging existing information reduces administrative burden, improves the member experience, and helps eligible individuals maintain coverage.

Domain 4: Analytics and Performance Measurement

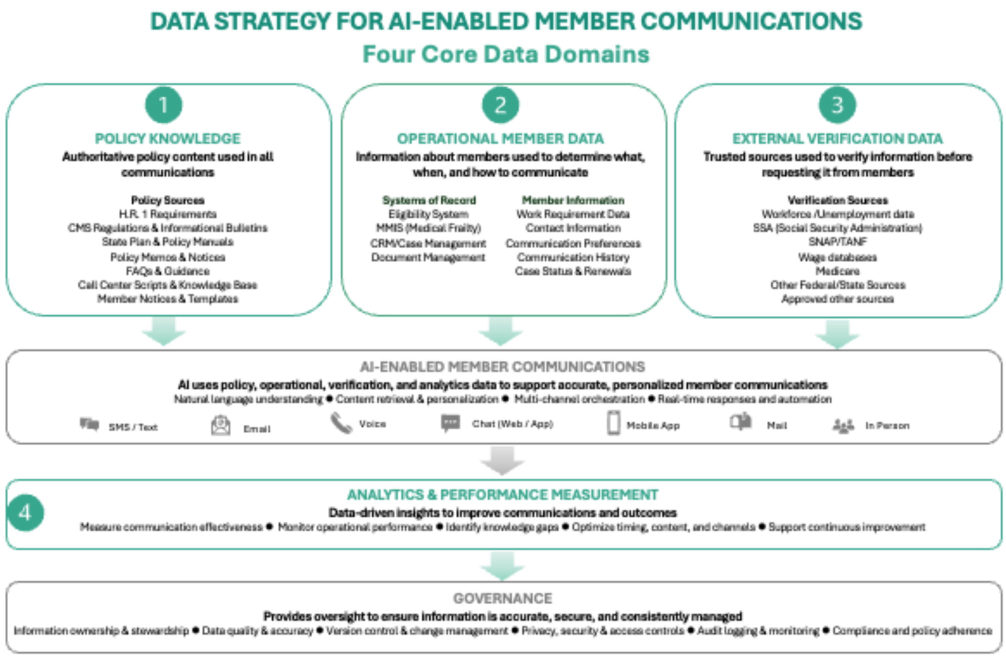
Performance measures should evaluate communication effectiveness, operational performance, and member outcomes. Analytics should identify opportunities to improve communication strategies, identify knowledge gaps, and support continuous improvement while preserving appropriate human oversight.

8.2 Governance

Governance ensures that information supporting member communications remains accurate, current, secure, and consistently managed. States should establish clear ownership, data stewardship, privacy protections, and processes that apply across every data domain, ensuring trusted information remains reliable.

8.3 Enterprise Data Architecture

The enterprise data architecture illustrates how the 4 core data domains work together to support AI-enabled member communications. Trusted policy knowledge, operational member data, and external verification data provide the information used to generate communications. Analytics measure communication effectiveness, while governance provides the enterprise oversight across all data domains.



8.4 State Readiness

Before implementing AI-enabled member communications, states should establish the foundational capabilities needed to support information management.

States should take on these activities as part of their AI process

- ☐ Identify authoritative policy sources
- ☐ Inventory the systems of record
- ☐ Assign ownership and stewardship for policy and operational information
- ☐ Establish enterprise AI governance
- ☐ Define an integration strategy
- ☐ Implement privacy, security, and audit controls
- ☐ Provide sufficient training for technology, operations, and policy staff
- ☐ Establish performance measures and processes for maintaining knowledge over time
- ☐ When applicable, assess and communicate AI's impact on the state workforce

As communication technologies continue to evolve, a data strategy provides a scalable foundation for innovation while preserving transparency, accountability, and the expertise of Medicaid professionals.

9. AI Model Training for Member Communications

The previous sections describe the use cases, guardrails, vehicles, and state readiness for AI-enabled member communications. Building on those recommendations, this section focuses on the standards that should govern how AI models are trained, validated, deployed, and continuously improved.

Unlike traditional software, AI systems learn from large collections of information and generate responses based on statistical patterns. As a result, the quality of training data directly influences the accuracy and reliability of member communications. Poorly trained models can generate incorrect policy interpretations, inconsistent guidance, or responses that unintentionally disadvantage certain member populations.

For HR1 implementation, model training should ensure that AI consistently produces responses that are grounded in approved Medicaid policy, understandable to members, equitable across populations, and aligned with evolving federal and state requirements.

The following recommendations build upon CMS Responsible AI Guidance, the CMS AI Playbook, the NIST Artificial Intelligence Risk Management Framework (AI RMF), FDA Good Machine Learning Practice principles, and emerging healthcare AI evaluation frameworks.

9.1 Train AI Models Using Approved Policy and Operational Resources

AI systems supporting Medicaid member communications should be grounded in the same authoritative resources used to train eligibility workers and customer service representatives. These resources may include CMS and state guidance, policy manuals, worker procedures, approved FAQs, member notices, contact center scripts, call trees, response libraries, escalation protocols, language standards, and accessibility guidance.

The specific training resources and validation path may vary by use case. For example, an internal policy-search tool may require approved manuals and FAQs, while a member-facing chatbot may also require plain-language standards, multilingual content, escalation rules, and responses to sensitive questions. States should define a use-case-specific training package for AI solutions. These training resources help align AI-generated communications with guidance provided by human staff and reduces inconsistencies across channels.

9.2 Configure and Train AI for the Intended Medicaid Use Case

Once the appropriate policy and operational resources have been identified, states should configure and train the AI system to perform its intended communication function. The training approach should be proportionate to the use case, audience, communication channel, and potential impact on members.

An internal policy-search tool, for example, may be trained primarily to retrieve and summarize approved guidance for state staff. A member-facing chatbot must also communicate in plain language, support appropriate languages and accessibility needs, recognize sensitive topics, and transfer members to human assistance when necessary.

Across use cases, training should emphasize the following behaviors:

Policy Fidelity	Clear and Accessible Communication	Language Quality	Conistant Application	Appropriate Escalation
• Accurately reflects state and federal policy	• Plain language at the 4th to 6th grade level	• Validated translations	• Reflects approved scripts, decision trees, and response libraries	• Routes to human when member is confused, distressed, or in coverage loss situation

9.3 Validate the AI System Before Deployment

Before an AI communication system is made available a structured validation process should be completed. Validation should be based on the system's intended use, audience, communication channel, and potential impact. It should assess the approved resources, prompts, retrieval configuration, response rules, integrations, and escalation workflows.

The state should define the acceptance criteria to determine whether the system is ready for deployment. The system vendor should conduct technical testing, correct identified defects, and provide evidence of performance. State policy, operations, communications, accessibility, and language experts should independently review representative outputs.

Establish the Validation Plan

The validation plan should be established before testing begins and should identify:

1. The intended use and boundaries of the AI system
2. The questions and scenarios the system is expected to handle
3. Required performance measures and acceptance thresholds
4. The state and vendor staff responsible for each test
5. The severity assigned to different types of errors
6. Conditions that require correction, retesting, or delayed deployment
7. The documentation required for state approval

Test scenarios should reflect real Medicaid interactions and include common questions, uncommon cases, sensitive topics, incomplete information, conflicting information, and intentionally ambiguous requests.

Validation Testing Types and Variables

Validation Type	Testing Variables
Technical and Policy Validation	• Factual accuracy against current federal and state policy • Grounding in approved source materials • Retrieval accuracy and source relevance • Citation or source-link verification • Consistency across repeated and differently worded questions • Hallucination testing using ambiguous, misleading, or unsupported requests • Appropriate refusal when approved information is unavailable • Prevention of disclosure or reproduction of sensitive information

Communication and Member Centered Validation	• Plain language and readability • Preservation of legal and policy meaning • Tone, clarity, and empathy • Multilingual accuracy and cultural appropriateness • Accessibility for members with disabilities • Suitability for the communication channel • Clarity of required actions, deadlines, and next steps
Operational and Human Hand-off Validation	• Correct routing to the appropriate program or staff unit • Escalation for sensitive, complex, or case-specific matters • Recognition of confusion, frustration, or repeated unsuccessful attempts • Appropriate refusal to make eligibility or exemption determinations • Successful transfer of the interaction and relevant context to a human agent • Integration with contact center, portal, CRM, and eligibility workflows • Consistent behavior across chat, SMS, voice, email, and agent-assist channels • Usability and effectiveness of the combined human–AI workflow
Equity Validation	• Preferred language • Disability and accessibility need • Reading and digital literacy level • Geographic location • Rural and urban access • Communication channel • Other relevant population characteristics permitted for evaluation

9.4 Classify, Correct, and Retest Failures

Validation should use a documented process for classifying and resolving defects. States may categorize failures as:

- **Critical:** Could cause loss of coverage, an incorrect eligibility conclusion, disclosure of protected information, or another material member harm
- **Major:** Provides incorrect or incomplete policy guidance, fails to escalate, or creates a substantial barrier to completing a required action
- **Minor:** Creates a readability, tone, formatting, or usability issue without changing the policy meaning

Corrective action should address the source of the failure. The remedy will vary based on the type and severity of the defect.

Identified AI problems and corrective actions

Incorrect policy answer	Correct or clarify the source material; adjust retrieval and response constraints; retest paraphrased questions
Inconsistent answers to similar questions	Refine prompts and approved response logic; expand the test-question library
Poor-quality translation	Engage qualified language reviewers; update glossaries and approved translations; repeat language testing
Failure to escalate a sensitive issue	Revise escalation triggers and routing logic; test realistic multi-turn conversations
Unsupported or incorrect citation	Correct document indexing and source mapping; verify citations against the generated response
Communication is too complex	Revise plain-language instructions and templates; repeat readability and member review

9.5 Validation Frequency

The frequency and depth of testing should reflect the potential impact on members. Low-risk internal tools may require a narrower review, while member-facing tools that address member coverage or actions should undergo more extensive and frequent validation.

Validation should occur:

- Before the initial pilot and/or production launch
- Frequently during the first weeks of a pilot and/or production launch
- Through ongoing review of flagged and randomly sampled interactions
- After changes to policy, prompts, scripts, call trees, models, data sources, languages, integrations, or communication channels
- Through periodic cross-functional reviews and broader recertification

9.6 Monitor Performance After Deployment

Validation should continue after deployment because policies, member questions, and system performance will change over time.

Monitoring should include:

- Flagged and randomly sampled communications
- Member and staff complaints or feedback
- Accuracy and hallucination events
- Escalation and transfer performance
- Unanswered or repeatedly misunderstood questions
- Readability, language, and accessibility performance
- Member outcomes, including task completion and avoidable call transfers
- Differences in performance across languages, channels, and populations

The purpose of monitoring is to both confirm technical performance and to determine whether AI communications improve member support and process efficiency.

9.7 Update, Revalidate, and Recertify AI Systems

States should establish a documented process for updating and reapproving AI systems when monitoring identifies a problem or when policies and operations change.

Revalidation Triggers

New CMS guidance or federal requirements
Changes to state Medicaid policy or worker procedures
Revised FAQs, scripts, call trees, or notices
Addition of a language, channel, or use case
Changes to the foundation model, vendor platform, or system integration
Significant declines in accuracy or escalation performance
Complaints, privacy incidents, or evidence of potential member harm

By planning for iterative testing, timely updates, and ongoing monitoring, states can improve AI communications as policies, member needs, and technologies evolve.

10. Multiple System Interconnectivity Standards

When integrating multiple systems, vendors, or AI-enabled tools, following federal security frameworks and interconnection processes can ensure data protection and compliance.

10.1 Core Federal Interconnectivity Standards

NIST SP 800-47

NIST SP 800-47 outlines requirements for system interconnection, including levels of system connectivity, required agreements, and needed security controls. These principles appear consistently in CMS and Medicaid ISRA documentation for state systems.

CMS Interconnection Security Agreement (ISA)

CMS requires an *Interconnection Security Agreement* for any system connection involving federally funded systems or data. This requirement is reflected in state's ISRA guidance, which notes that CMS-certified systems must maintain documented interface characteristics, security expectations, and data-sharing specifications.

ARC-AMPE (formerly MARS-E)

State Medicaid agencies that administer ACA-related eligibility and enrollment functions, particularly those operating systems connected to the CMS Federal Data Services Hub must comply with Acceptable Risk Controls for ACA, Medicaid, and Partner Entities (ARC-AMPE) controls. These controls govern authentication, authorization, auditing, boundary protection, communications security, and other areas. Systems must explicitly document ARC-AMPE adherence within their security frameworks. Systems must document compliance with applicable ARC-AMPE controls within their security frameworks. Unresolved noncompliance can jeopardize continued federal approval and funding associated with affected Medicaid systems, making timely remediation of identified deficiencies essential.

10.2 Requirements Related to SSA-Certified Systems

If a state's system is SSA-certified, there are additional requirements for interconnection:

- **SSA Change Notification** within 90 days when a state connects a new system, tool, AI agent, or interface that accesses SSA data.
- **Security Impact Analysis (SIA)** approval is required before changes are made to connectivity or architecture for most states and systems.
- **Privacy Impact Assessment (PIA)** updates and approval is required before changes are made to connectivity or architecture for most states and systems.

10.3 Considerations Before Connecting New Tools or Vendors

State agencies should conduct several steps before approving new system connections:

Security Impact Analysis (SIA)
Completed and approved by the agency's cybersecurity team

Architecture updates
Including current diagrams showing new interfaces, data flows, and the connectivity point(s)

Assessment of data sharing with AI or external vendors
Reviewed and approved by the agency's security team, considering federal restrictions on PHI, PII, FTI, and SSA data.

Review of applicable ARC-AMPE controls
Reviews may include AC-04, AC-06(02), AC-17, AC-18, AC-20(01), CA-03, CA-09, CA-09(01), IR-06, PL-02, SC-07(03), SC-07(05), SI-04

10.4 Authority to Connect (ATC)

Depending on the system involved:

- **CMS-certified systems** may require an Authority to Connect (ATC).
- **Non-CMS systems** may not require an ATC, but internal agency security approval is still required.

CMS required system documentation such as the System Design Document, Configuration Management Plan, PIA, ISRA, and Data Management emphasize strict interconnection requirements and annual updates to demonstrate continued compliance.

10.5 Annual Control Updates and POA&Ms

Historically known as **MARS-E controls**, now **ARC-AMPE**, states must update these controls annually for CMS-certified systems. Failure to meet controls results in findings that become **Plan of Action and Milestones (POA&Ms)** that must be remediated.

10.6 Key Operational Realities for Integrating Multiple Vendors or AI Tools

- System connectivity changes may take **several months or longer** depending on the state's platform and required security approvals
- Agencies must validate whether external partners (e.g., education institutions, community service organizations) can provide required verification data before connecting them to eligibility or reporting systems
- Changes impacting eligibility systems often require updates to business rules, job aids, policy documents, and training.

Integrating multiple systems, vendors, or AI tools can be complex and require annual updates. States should plan set timelines to ensure coordination across security, architecture, and operational teams.

11. Benefits and Risks of Using AI for Member Outreach

With the many outreach and notification requirements set forth in HR1, AI offers states a way to extend outreach capacity and meet compliance deadlines, but it also introduces risks that must be carefully weighed. This section outlines the primary benefits and risks associated with using AI for member outreach and offers considerations to help states make an informed decision on whether to move forward with an AI approach to member outreach.

Multi-channel Outreach	Incorrect Information at Scale
Expanded Outreach Capacity	Public Perception and Distrust
Reduced Call Center Volume	State Labor Law and Bargaining Requirements
Rapid Updates as Policies Evolve	AI Hallucinations or Inappropriate Content
Potential Cost Savings	PHI/PII Exposure
Freeing Staff for High-Impact Work	

11.1 Benefits of Using AI for HR1 Outreach

AI tools can extend a state's reach and speed without requiring a proportional increase in staff. Key benefits include:

1. **Multi-channel outreach:** AI can support simultaneous, coordinated outreach across email, text, and phone, helping ensure members receive time-sensitive notices through the channel most likely to reach them. Rather than sequencing outreach one channel at a time, AI can trigger a coordinated sequence, such as an initial email, followed by a text reminder, and then an automated call if a member has not engaged, which increases the likelihood that a critical redetermination or coverage notice is seen and acted upon.
2. **Expanded outreach capacity:** AI allows states to reach significantly more members without straining already limited communications staff. A single outreach campaign can be scaled to hundreds of thousands of members at once, which is particularly valuable given the compressed timelines under HR1 and the reality that many state communications teams do not have the staffing to manually manage outreach at scale.
3. **Reduced call center volume:** AI-driven chatbots and virtual agents can field routine member questions, such as status checks, document requirements, or deadline reminders, reducing the number of calls that live call center staff must handle. This can shorten wait times for members with more complex issues and help states avoid the need to rapidly hire and train large numbers of temporary call center staff to meet a surge in demand.
4. **Rapid updates as policy evolves:** As HR1 requirements, guidance, or state policy change, AI content sources (such as a chatbot's underlying knowledge base or a messaging template library) can be updated centrally and quickly. This avoids the lengthy re-training, re-printing, or manual re-scripting process that would otherwise be needed to update thousands of individual staff or paper materials, helping ensure members consistently receive current and accurate information.

5. **Potential cost savings:** By automating high-volume, repetitive outreach tasks, AI has the potential to lower the overall cost of the Medicaid outreach program relative to scaling up staff, printing, and postage to meet the same demand. Savings can also come from reduced overtime, reduced reliance on temporary staffing agencies, and fewer costly outreach errors caused by manual data entry or fatigue.
6. **Freeing staff for high-impact work:** AI can absorb routine, high-volume outreach tasks, allowing call center and communications staff to focus on complex, nuanced, or high-risk member interactions that require human judgment, such as members with disabilities, limited English proficiency, or unstable housing. This can improve both staff morale and outcomes for the members who need the most individualized support.

11.2 Risks of Using AI for HR1 Outreach

These benefits must be weighed against risks that could undermine member trust, compliance, or program integrity if not properly managed. Key risks include:

1. **Incorrect information at scale:** Because AI can reach large numbers of members quickly, an error in content, logic, or an underlying data source can distribute incorrect information widely before it is caught. Unlike a single staff error, a flawed automated campaign could reach thousands of members with the same mistake within minutes or hours, potentially causing missed deadlines, confusion, or unnecessary calls to the state, and requiring a costly, time-sensitive correction campaign.
2. **Public perception and distrust:** Some members may be fearful or distrustful of AI-driven communications, particularly given broader public concerns about AI making decisions that affect access to benefits. Members may disregard, delete, or fail to respond to messages they perceive as automated or impersonal, or may distrust the accuracy of information coming from a chatbot rather than a person, which could ultimately reduce engagement rather than increase it.
3. **State labor law and bargaining requirements:** Where applicable, state labor laws and collective bargaining agreements may restrict or require negotiation before AI can be used to perform work previously done by state staff, including outreach or call center functions. States that move forward without first engaging affected unions or complying with applicable notice and bargaining obligations may face grievances, delays, or legal challenges that could disrupt implementation timelines.
4. **AI hallucinations or inappropriate content:** AI tools, particularly generative or conversational systems, can produce inaccurate, fabricated, or inappropriate responses, sometimes referred to as hallucinations. This is especially concerning when members rely on that information to maintain coverage, understand eligibility requirements, or meet a deadline, since acting on incorrect guidance could result in a member losing coverage through no fault of their own.
5. **PHI/PII exposure:** Improper configuration, access controls, or data handling within AI systems could expose protected health information (PHI) or personally identifiable information (PII), resulting in a reportable breach under HIPAA or state law. Risk can be

heightened when AI tools are integrated with multiple data sources, or when member data is used to train or fine-tune a model without adequate safeguards.

States should weigh the benefits and risks described above rather than adopting AI by default.

11.3 Guide for States Assessing the Use of AI for Communications

State AI Risk Tolerance	Include any state laws or policies that may prevent or limit the use of AI for member outreach.
Prior AI Experience	Whether the state has used AI previously, and whether member outreach is the right first use case to expand or introduce AI adoption.
IT Team Capacity	Whether IT staff have the bandwidth to implement and support an AI outreach solution given competing HR1 priorities
Member Acceptance	The degree to which members may fear or resist interacting with AI or an AI agent.
Staff and Member Education Needs	The need to educate both state staff and members on what AI tool is being used and what data informs it, including clear disclaimers.
Labor agreements	The need to comply with union and other labor agreements with respect to communicaitons and the use of AI.
Availability of Misinformation Safeguards	The availability of ways to detect and prevent the spread of misinformation generated or distributed by AI tools.
Ongoing Monitoring Needs	The need to monitor member awareness and understanding, including through third-party platforms, to gauge how outreach is landing with members.

12. Summary

HR1 community engagement requirements create significant communications and operational challenges for state Medicaid agencies. With millions of potentially affected members, compressed timelines, and new verification requirements, states need scalable ways to provide timely, accessible, and consistent outreach. AI-enabled communications can expand capacity, personalize member support, and reduce pressure on contact centers while augmenting—not replacing—Medicaid staff.

Effective implementation requires rigorous planning, testing, monitoring, governance, and controls. States should determine where AI is appropriate based on their needs, capabilities, and risk tolerance. When implemented responsibly, AI can strengthen member communications, support completion of required actions, and help reduce avoidable coverage loss while preserving human judgment and accountability. Ultimately, members should remain at the center of whatever outreach approach a state selects

Appendix A. Selected Sources and Implementation Resources

Applicability of the Sources

CMS and NIST resources provide the primary public-sector references for the lifecycle, governance, testing, and monitoring practices discussed in this section. FDA, WHO, and academic healthcare AI frameworks provide useful safety and evaluation practices but generally do not constitute direct regulatory requirements for Medicaid communication systems. States should apply each source according to the use case, applicable law, agency policy, and potential impact on members.

Primary CMS and Federal Guidance

Centers for Medicare & Medicaid Services. CMS AI Playbook, Version 4.
Provides practical guidance on selecting AI use cases, defining state and vendor roles, conducting proofs of concept and pilots, establishing performance measures, managing change, and monitoring systems after deployment.

Centers for Medicare & Medicaid Services. Guidance for Responsible Use of Artificial Intelligence at CMS.
Describes CMS expectations for secure and responsible AI use, including suitability assessment, protection of sensitive information, human oversight, transparency, and risk management.

National Institute of Standards and Technology. Artificial Intelligence Risk Management Framework (AI RMF 1.0).
Organizes AI risk management into the Govern, Map, Measure, and Manage functions. It is particularly useful for assigning responsibilities, defining risks, establishing evaluation criteria, and documenting corrective action.

National Institute of Standards and Technology. Artificial Intelligence Risk Management Framework: Generative Artificial Intelligence Profile, NIST AI 600-1.
Extends the AI RMF to generative AI risks, including inaccurate or fabricated content, data privacy, information integrity, harmful bias, human overreliance, and the need for ongoing testing and monitoring.

Healthcare AI Safety and Lifecycle Guidance

U.S. Food and Drug Administration, Health Canada, and United Kingdom Medicines and Healthcare products Regulatory Agency. Good Machine Learning Practice for Medical Device Development: Guiding Principles.
Although designed for AI-enabled medical devices, these principles provide useful practices for representative data, independent testing, human–AI performance, clear user information, and lifecycle monitoring.

U.S. Food and Drug Administration, Health Canada, and United Kingdom Medicines and Healthcare products Regulatory Agency. Predetermined Change Control Plans for Machine Learning-Enabled Medical Devices: Guiding Principles.
Provides an informative model for defining anticipated system changes, protocols for

implementing those changes, evidence required for revalidation, and limits beyond which additional review is necessary. These concepts are particularly useful for vendor contracts and recertification plans.

World Health Organization. Ethics and Governance of Artificial Intelligence for Health: Guidance on Large Multi-Modal Models.
Addresses generative AI use in patient-facing services, healthcare administration, education, and research. The guidance emphasizes transparency, stakeholder participation, post-deployment review, risk assessment, and protection from misleading or harmful outputs.

Healthcare AI Evaluation and Reporting Frameworks

Vasey, B., et al. Reporting Guideline for the Early-Stage Clinical Evaluation of Decision Support Systems Driven by Artificial Intelligence: DECIDE-AI. Nature Medicine, 2022.
Supports small-scale, real-world evaluation before broad deployment. Its attention to workflow, human factors, user behavior, system failures, and iterative improvement is relevant to controlled Medicaid AI pilots.

Collins, G. S., et al. TRIPOD+AI Statement: Updated Guidance for Reporting Clinical Prediction Models That Use Regression or Machine Learning Methods. BMJ, 2024.
Provides standards for transparent documentation of model purpose, data, development, validation, performance, limitations, and updates. It is most directly applicable when a Medicaid AI system includes predictive or prioritization models.

Liu, X., et al. Reporting Guidelines for Clinical Trial Reports for Interventions Involving Artificial Intelligence: The CONSORT-AI Extension. Nature Medicine, 2020.

Rivera, S. C., et al. Guidelines for Clinical Trial Protocols for Interventions Involving Artificial Intelligence: The SPIRIT-AI Extension. Nature Medicine, 2020.
CONSORT-AI and SPIRIT-AI are most relevant when states or research partners conduct formal prospective evaluations of AI interventions. They emphasize advance definition of the intervention, human–AI interaction, error handling, evaluation outcomes, and changes made during testing.

Organizational and Vendor Management Standards

International Organization for Standardization. ISO/IEC 42001:2023—Artificial Intelligence Management System.
Provides requirements for establishing, operating, maintaining, and continually improving an organizational AI management system. States may use it as a reference for enterprise governance and vendor due diligence.

Organization for Economic Co-operation and Development. OECD AI Principles.
Provides a values-based framework for trustworthy AI, including human-centered design, transparency, robustness, safety, accountability, and respect for rights.

5. State-Specific Training and Validation Resources

External standards should be supplemented by the state's own approved operational materials. Depending on the use case, these may include:

- Current CMS and state Medicaid policy guidance
- Eligibility-worker manuals and desk procedures
- Contact center training materials
- Approved scripts and response libraries
- Call trees, routing instructions, and escalation protocols
- Member notices, FAQs, and communication templates
- Language-access glossaries and approved translations
- Plain-language and accessibility standards
- De-identified call logs, complaints, appeals, and recurring member questions
- Existing quality-assurance criteria and contact center review forms

These materials define how the state interprets policy and communicates with members in practice. Section 8 further describes the policy, eligibility, verification, contact, preference, and engagement data sources that may support AI-enabled outreach.

References

Centers for Medicare & Medicaid Services. (2026). Medicaid Community Engagement Requirement for Certain Individuals Interim Final Rule with Comment Period (CMS-2454-IFC).

Centers for Medicare & Medicaid Services. (2022). Minimum Acceptable Risk Standards for Exchanges (MARS-E), Version 2.2. https://www.cms.gov/CCIIO/Resources/Regulations-and-Guidance/Downloads/2-2-MARS-E-Suite-of-Documents.zip

Centers for Medicare & Medicaid Services. (2025, December 8). CMCS informational bulletin: Community engagement requirements.

Colorado Department of Health Care Policy and Financing. (2026). H.R. 1 Medicaid partner frequently asked questions.

Iowa Department of Health and Human Services. (2025). Medical assistance text messaging program.

U.S. Department of Health and Human Services. (2025, December 4). HHS unveils AI strategy to transform agency operations.

Center for Health Care Strategies. (2026). A summary of national Medicaid work requirements.